

## INTERFERON ALFACON-1 -> immunodeficiency 45

For clinical-collaborator outreach.

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### *Why this hypothesis is real*

Drug-target convergence on the disease scores **0.62** (noisy-OR over OT target-disease associations). This pair is fully novel relative to all known drug-indication assignments in ChEMBL. Therapeutic direction is **aligned** with OT genetic evidence (the drug pushes the target the disease-protective way). Lead target is well expressed in the disease-relevant tissue (blood). Orthologous-gene model-organism evidence is strong (phylo\_score 0.55, n=1). Reactome co-membership reinforces the indirect mechanism with **3** shared specific pathways.

### *Why now (IP runway + literature footprint)*

The substance is a biologic or otherwise outside the FDA Orange Book; check the Purple Book and direct registry for IP runway.

### *Clinical opportunity*

US prevalence figures are not in the curated map; for an orphan condition this is expected. The KOL can size the addressable population.

### *What we propose*

**BioBix** (Department of Mathematical Modelling, Statistics and Bioinformatics, Ghent University) brings bioinformatics support to the collaboration:

- Target validation across blood / immune / cancer multi-omics datasets
- Biomarker candidate identification from public + private cohorts
- Patient stratification analytics for trial-enrichment design
- Mechanism-of-action mining via systems-biology priors
- Quick-turn evidence syntheses for IIT and partnered programs

Proposed collaboration model:

- Investigator-initiated trial framework with the substance owner
- BioBix as bioinformatics analytics partner (target/biomarker/biostatistics)
- Joint publication strategy; data-sharing agreement gated on IP terms

### *Next step*

A 30-minute scoping call to align on feasibility. We would come prepared with a brief mechanistic dossier, an annotated literature pull, and a pilot biomarker shortlist for the disease.

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